

CLINICAL EVIDENCE BRIEF

A direct answer, with every material finding attached to its source.

RESEARCH QUESTION

Summarize the efficacy, safety findings, and limitations of this treatment. MAIN GOAL IS TO CHECK IF THERE IS ANY DRUG INTERACTIONS THAT ARE HARMFUL

4 SOURCES

91% SOURCE SUPPORT

LOCAL EXTRACTION

JUL 15, 2026

DIRECT ANSWER

Yes. The sources identify several medication-related concerns. The strongest concern is cumulative QT-prolongation risk, especially where QT-active medications overlap with a prolonged QTc or electrolyte abnormalities. Frequent acid suppression may also reduce oral iron absorption and delay recovery. Propranolol may worsen orthostatic symptoms or blunt compensatory tachycardia. Ibuprofen may add bleeding-related risk when anemia or heavy bleeding is present. These findings describe documented risk signals, not proof that a harmful event occurred.

FINDINGS THAT ANSWER THE QUESTION

- Medication-related QT effects are also relevant because hydroxychloroquine can prolong the QT interval.
[6]
- Hydroxychloroquine + recent azithromycin exposure: Moderate concern. Both can prolong QT.
[9]
- Hydroxychloroquine + borderline QTc 477 ms: Clinically relevant. Repeat ECG and review modifiable risk factors, including electrolytes.
[9]

- Propranolol + orthostatic symptoms: Moderate concern. It could worsen lightheadedness or reduce the compensatory heart-rate response.
[9]
- Omeprazole + oral iron: Low-to-moderate concern. Frequent acid suppression may reduce iron absorption; actual use frequency is uncertain.
[9]
- Ibuprofen + heavy menstrual bleeding or anemia: Moderate concern. NSAID use may contribute to gastrointestinal blood loss.
[9]
- However, iron stores remain depleted and heavy menstrual bleeding continues, so relapse risk remains substantial.
[2]
- Heavy menstrual bleeding is likely, but occult gastrointestinal loss has not been formally excluded. Will ferritin normalize with oral therapy alone?
[2]

SAFETY FINDINGS

- Symptom Response Symptom Baseline Current Fatigue Severe, daily Moderate; about 40% improved Palpitations 5-15 minutes several times weekly Brief episodes once or twice weekly Exertional dyspnea With one flight of stairs Only with faster walking or two flights Lightheadedness Frequent on standing Occasional Menstrual bleeding Heavy for 6-7 days Still heavy; no
[1]
- The reduction in QTc may reflect improved electrolytes, lower heart rate, medication changes, or normal test variability.
[2]
- Repeat ECG after electrolyte optimization QTc may vary and Bazett correction can overestimate at higher rates.
[3]
- Medication review Avoid additional QT-prolonging drugs where possible.
[3]

WHAT REMAINS UNCERTAIN

- It does not prove that hydroxychloroquine caused the initial QTc finding.
[2]
- Unresolved Questions Question Why It Matters What is the definitive source of blood loss?
[2]
- The note does not establish a definitive cardiac diagnosis.
[7]
- Recommended Medication Actions Action Priority Hold non-prescribed propranolol until the prescriber reviews blood pressure, ECG, and symptom pattern.
[10]

FOLLOW-UP QUESTIONS

- Has QTc remained within range after electrolyte correction and medication changes?
- What is the confirmed frequency and timing of acid-suppressing medication relative to oral iron?
- Did orthostatic symptoms improve after propranolol was stopped?
- Has gastrointestinal or menstrual blood loss been adequately evaluated?

SOURCE PASSAGES

These excerpts are included for verification. They are not additional conclusions.

[1] 04_follow_up_and_treatment_response.pdf, page 1

Symptom Response Symptom Baseline Current Fatigue Severe, daily Moderate; about 40% improved Palpitations 5-15 minutes several times weekly Brief episodes once or twice weekly Exertional dyspnea With one flight of stairs Only with faster walking or two flights Lightheadedness Frequent on standing Occasional Menstrual bleeding Heavy for 6-7 days Still heavy; no

[2] 04_follow_up_and_treatment_response.pdf, page 2

However, iron stores remain depleted and heavy menstrual bleeding continues, so relapse risk remains substantial.

Heavy menstrual bleeding is likely, but occult gastrointestinal loss has not been formally exclude Will ferritin normalize with oral therapy alone?

The reduction in QTc may reflect improved electrolytes, lower heart rate, medication changes, or normal test variability.

It does not prove that hydroxychloroquine caused the initial QTc finding.

Unresolved Questions Question Why It Matters What is the definitive source of blood loss?

[3] 02_laboratory_and_ecg_report.pdf, page 2

Repeat ECG after electrolyte optimization QTc may vary and Bazett correction can overestimate at higher rates.

Medication review Avoid additional QT-prolonging drugs where possible.

[6] 01_patient_clinical_summary.pdf, page 2

Medication-related QT effects are also relevant because hydroxychloroquine can prolong the QT interval.

[7] 01_patient_clinical_summary.pdf, page 2

The note does not establish a definitive cardiac diagnosis.

[9] 03_medication_safety_review.pdf, page 1

Hydroxychloroquine + recent azithromycin exposure Moderate concern Both can prolong QT. Azithromycin course ended about 10 days before ECG, so

Hydroxychloroquine + borderline QTc 477 ms Clinically relevant Supports repeat ECG and review of modifiable risk factors, including magnesium

Propranolol + orthostatic symptoms Moderate concern Could worsen lightheadedness or reduce compensatory heart-rate response.

Omeprazole + oral iron Low-to-moderate concern Frequent acid suppression may reduce iron absorption; actual use frequency is

Ibuprofen + heavy menstrual bleeding / anemia Moderate concern NSAID use may contribute to gastrointestinal blood loss, although no overt GI bleed

[10] 03_medication_safety_review.pdf, page 1

Recommended Medication Actions Action Priority Hold non-prescribed propranolol until the prescriber reviews blood pressure, ECG, and symptom pattern.